



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نصره عبدالباسط امام السيد
المحافظة : الجيزه الحي / المدينة : الجيزه
رقم البطاقة : 26102132100361 رقم التليفون : 01098099805
العمر (تاريخ الميلاد) : 13-02-1961 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نصرة عبدالباسط امام السيد
National ID: 26102132100361



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



١٥٨٨
اسمها فاروقه



جمهورية مصر العربية
مادة تحقيق الشخصية

نصره

عبدالباسط امام السيد

١٠ العظه الفريش الملاحين

الجيزة - الجيزة

١٩٦١/٠٢

٢٦١.٢٦٣٢١.٠٠٣٦١

LB8026686

Patient's Name: Nasra Abdelbasset Imam Al-Sayed
Date: 2026-06-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTIDETECTOR CT EXAMINATION OF THE CHEST, ABDOMEN, AND PELVIS
WAS PERFORMED FOLLOWING INTRAVENOUS CONTRAST**

Chest

- Postsurgical changes related to prior right-sided mastectomy are noted.
- No evidence of recurrent or residual enhancing soft tissue mass within the right mastectomy bed.
- No abnormal chest wall soft tissue lesion identified.
- No pathologically enlarged axillary, supraclavicular, mediastinal, or hilar lymph nodes.
- Bilateral infrascapular soft tissue masses characteristics of elastofibroma dorsi.
- No focal pulmonary mass or suspicious pulmonary nodule.
- No focal air-space consolidation.
- No pleural effusion or pneumothorax.
- Mild cardiomegaly is noted.
- No pericardial effusion.
- Thoracic aorta and major mediastinal vascular structures are unremarkable.



Abdomen

- A focal hepatic lesion demonstrating peripheral enhancement is identified at segment VII , measuring approximately 1.2 x 0.8 cm. The lesion remains indeterminate on the current examination and may represent a metastatic deposit, atypical hemangioma, or other enhancing hepatic lesion. Correlation with prior studies and further characterization with dedicated contrast-enhanced MRI liver is recommended.
- No additional focal hepatic lesions identified.
- No intrahepatic or extrahepatic biliary duct dilatation.
- Gallbladder is unremarkable.
- Pancreas demonstrates normal morphology and enhancement.
- Spleen is normal in size and attenuation.
- Adrenal glands are unremarkable.
- Both kidneys demonstrate normal size and enhancement without focal solid lesions or hydronephrosis.
- No abdominal lymphadenopathy.
- No ascites, peritoneal thickening, or omental nodularity.

Pelvis

- Urinary bladder appears unremarkable.
- Uterus and adnexal structures are grossly unremarkable on CT appearance.
- No pelvic lymphadenopathy.
- No free pelvic fluid.
- No evidence of pelvic soft tissue mass.

Name: نصره عبد الباسط امام السيد		
MR. : 6601		
Detailed Medical Report		
		السيد
تحية طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
نشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was off systemic therapy for eight months following a CT-guided core biopsy and bone scan (BS) that revealed a right-sided capillary lesion. The patient presented today for initiation of ARAMIDIX and supportive therapy.		
The patient has metastatic breast cancer involving the brain, bones, and liver while on adjuvant aromatase inhibitor therapy. The multidisciplinary team decided to initiate first-line endocrine therapy with Faslodex (fulvestrant) and Ibrance (palbociclib) due to endocrine resistance, pending laboratory work-up to commence treatment.		
The patient had not yet received therapy; the most recent CT scans were from November 2025. New CT imaging and a request for re-approval of Palbociclib were planned.		
The patient has metastatic breast cancer to the brain and bone. History since 2021 includes neoadjuvant chemotherapy (AC/TXT) followed by right mastectomy with ypT1c, 3/21 lymph nodes, and postoperative radiation plus aromatase inhibitor (Femara). In October 2025, MRI brain revealed a left cerebellar mass confirmed by CT in November 2025. The patient received whole-brain irradiation (WBI) and a CT of the chest/abdomen/pelvis in November 2025 showed a right hilar lymph node 1.6 × 1 cm. A biopsy attempt failed due to the lesion's small size. Bone scan in November 2025 demonstrated an active focal lesion in the distal shaft of the femur. The multidisciplinary team on April 2nd recommended Faslodex/IBRANCE with palliative radiotherapy if high fracture risk. Today the patient received C1D15 Faslodex and is awaiting follow-up MRI of the brain as the patient reports dizziness.		
The patient complained of dizziness and MRI of the brain had not yet been performed. The patient was scheduled to receive Ibrance and Faslodex after laboratory results; the next visit was planned for one month later.		
Today the patient received C1D28 Ibrance; the previous dose was not received. Current laboratory values were TLC/ANC 2.13/1.02.		
Today the patient received Faslodex only after laboratory results; ANC was 0.6, prompting a hold for one week.		
The patient presented for Ibrance as the previous CBC showed neutropenia. A new CBC on 13/7 showed TLC/neu 2.3/0.9. A new CBC on 13/7 showed TLC/neu 3/1.4, allowing initiation of Ibrance 100 mg for one month. Dental assessment was performed.		
The patient has metastatic breast cancer to the brain, liver, and bone with HER2-negative status. History includes right breast cancer treated with neoadjuvant chemotherapy, right mastectomy (ypT1c, ypN1), and radiation. The patient received Arimidex from December 2021 until October 2021,		
تحريراً في ,		
التاريخ :	19-09-2026	الوقت :
الطبيب المعالج :	Reham Ahmed Fathy Ibrahim	
يعتمد ,		

followed by whole-brain irradiation in December 2025. CT scans in November 2025 showed a right hilar lymph node 7 mm and a bone scan revealed a left femoral shaft lesion. The patient has been on Faslodex and Ibrance since May 2026 and is receiving zoledronic acid.

Patient's Name: Nasra Abdelbasset Imam Al-Sayed
Date: 2026-06-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Osseous Structures

- Solitary expansile lytic lesion involving the left third rib with no associated cortical destruction.
- No additional definite destructive osseous lesions identified on the current examination.
- Mild multilevel degenerative spondylotic changes of the visualized spine.
- No pathological fracture detected.

Impression:

- Postsurgical changes of prior right-sided mastectomy with no CT evidence of recurrent or residual local tumor.
- Indeterminate peripherally enhancing focal hepatic lesion; further characterization with dedicated MRI liver is recommended. Metastatic disease cannot be excluded.
- Solitary lytic lesion involving the left third rib, suspicious for osseous metastatic deposit.
- Mild cardiomegaly.

Yours,

Dr. Yahya Khalifa ,Msc,FEBR

Patient Name: Nasra Abd Al Baset Emam Al Sayed	Date : 20/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 64 Year(s)
Medical ID: 1519/2021	Pat. ID: 17011

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of managed right breast cancer for metastatic workup , no available previous examination

I- CHEST:

- Right MRM showed clear operative bed with no evidence of enhanced focal lesion or fluid collection.
- Bilateral basal lung lobes atelectatic bands are noted.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- The heart is seen enlarged in size.
- Left 4th rib healed fracture is noted
- Small hiatus hernia.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
 - The right hepatic lobe segment VII showed ring enhanced focal lesion measures about 1.6x1 cm
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Apart from right lower cortical simple cyst measures about 6mm
- Bilateral bulky suprarenal glands
- Normal C.T appearance of the spleen, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined bones showed dorsolumbar spondylosis with no worrisome lytic or sclerotic lesions.

Patient Name: Nasra Abd Al Baset Emam Al Sayed	Date : 20/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 64 Year(s)
Medical ID: 1519/2021	Pat. ID: 17011

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE BRAIN, CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of managed right breast cancer for metastatic workup , no available previous examination

I- BRAIN:

- Left cerebellar space occupying lesion is seen measures about 3.2 x 2.3 cm in axial dimensions showing thick marginal enhancement and surrounded by perifocal edema and mass effect in the form of compression of the posterior aspect of brain stem.
- Moderate dilatation of supra-tentorial ventricular system with no significant midline shift or deformity.
- Prominent cortical sulci and extra axial CSF spaces.
- Preserved normal gray-white matter interface.
- No extra-axial or intra-cerebral areas of fresh blood densities.

OPINION:

- *Left cerebellar SOL likely metastatic.*
- *Brain involutinal changes with no evidence of ischemic vascular insults.*

II- CHEST:

- Right MRM showed clear operative bed with no evidence of enhanced focal lesion or fluid collection.
- Bilateral basal lung lobes atelectatic bands are noted.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- The heart is seen enlarged in size.
- Left 4th rib healed fracture is noted
- Small hiatus hernia.

III- ABDOMEN AND PELVIS:



Pathology Report

Name: نصره. عبد الباسط امام السيد

Age: 60 years.

Clinical data: Right Breast mass

Specimen type: Tru - cut biopsy.

Date (Received): 6/4/2021

Date (reported): 13/4/2021

Lab. Number: 731/21

Hospital Number: 21/مباريه

Gross Description:

Five grayish white tissue cores up to 1.5cm, totally submitted.

Sections prepared from the paraffin block for immune-staining using Benchmark ULTRA system for: ER clone (SP1), PR clone (1E2), Her2 clone (4B5).

Microscopic Examination:

histopathological examination of sections prepared from the tru cut biopsy received reveals; cores of an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stroma! desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].

ER: Positive strong nuclear staining in 70% of the tumor cells.

PR: Positive moderate nuclear staining in 70 % of the tumor cells.

Her2: No membranous staining of the tumor cells.

Diagnosis:

Right breast mass, core biopsy:

- Invasive duct carcinoma, grade II.
- ER: Positive (8/8)
- PR: Positive (7/8)
- Her2: Negative (score 0).

Prof. Dr. Amal Hareedy.

Dr. Marwa Mahmoud.

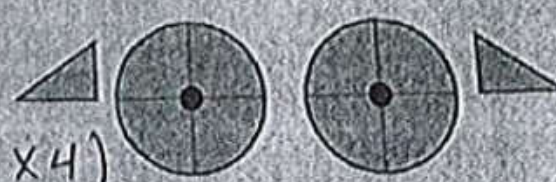
Dr. Mona Mohamad

MDT FORM

4/12/2020

Name	نخلة عبد الباق	MRN	10191C1
Age	70	ID	DMS 77.1
Menopausal status	Post	Phone	01554213372
RS SCORE		Family history	C71.012C1.011
MI		REASON OF MDT	
Comorbidity			

Clinical Data

				R	L
8/8 +ve	PR: 7/8 +ve	HER 2: -ve	KI67:		
Mammography:					
2021: NACTH (AC x4 + TXT x4) RT MRI TIC N3/21					
Workup:					
2021: PORTH AI (Femora)					
2025: mri brain → It cerebellar mass (mets)					
Recommendations:					
2025: CT CAP → Rt HFL (1.6 x 1) + bil. bulky supra renal					
→ bx → Too small for interventional					
2025: Bone Scan → active Focal Lesion in distal shaft of Femur					

OUTCOME & PLAN

For many

for COX 416 Ibuprofen + Paracetamol
± Palliat. Radiation on femoral shaft
if ↑ risk of fracture

SIGNATURE

ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiat
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Patient Name: Nasra Abd.Al Baset Emam Al Sayed	Date : 01/10/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 64 Year(s)
Medical ID: 1519/2021	Pat. ID: 17011

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1 WIs
- Post contrast series

MR Findings:

- A space occupying soft tissue mass is seen at the left cerebellum measures about 3.5 x 2.3 cm in axial dimensions being of low T1, high signal in T2/FLAIR and surrounded by perifocal edema. It exerts mass effect in the form of compression of the brain stem as well as compression and attenuation of 4th ventricle.
- Bilateral focal areas of altered signal intensity are noted at the cerebral subcortical & deep periventricular white matter eliciting patchy high T2 and FLAIR signals while they are inconspicuous to hypointense signal in the T1 Series. No perifocal edema or notable mass effect detected.
- Bilateral periventricular thin sheets of faint high T2/FLAIR signals with no surrounding edema or associated mass effect.
- Prominent cortical sulci, sylvian fissures and extra-axial CSF spaces as well as the cerebral ventricular system.
- Normal grey / white matter differentiation.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

- Left cerebellar mass? Metastatic, for correlation with previous studies.
- Mild arterio-sclerotic leuco-encephalopathy with bilateral cerebral ischemic foci.
- Mild age related cerebral involutional changes.

Signature Prof. Ali Remah

23/10/2025 05:31 م

Signature Prof. Ali Remah

23/10/2025 05:34 م

Name NASRA ABDELBASET EMAM

Date : 20 November 2025

**DUPLEX EXAMINATION OF THE VENOUS SYSTEM OF
THE RIGHT UPPER LIMB REVEALED: -**

Deep Venous System: -

- The IJV subclavian, axillary, rest of brachial veins down to radioulnar veins all are patent, totally compressible with normal color filling & preserved respiratory variation.

Superficial Venous System: -

- Patent cephalic & basilic vein
- No edema

OPINION: -

No DVT nor superficial thrombophlebitis.

*Best Regards,
Dr/Abubak Hejazi MSc FEBR*

Name : Nasra Abdelbasset Imam ElSayed

Date : 26-11-2025 01:43

Branch : Dokki

Medical No. : 259266

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 64-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Active focal lesion is seen at the distal shaft of left femur.
- Lt. 4th rib lateral aspect focal increased tracer uptake; likely traumatic.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Active focal lesion is seen at the distal shaft of left femur; for more radiological correlation.
- The rest of skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name: Nasra Abd Al Baset Emam Al Sayed	Date : 20/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 64 Year(s)
Medical ID: 1519/2021	Pat. ID: 17011

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE BRAIN, CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of managed right breast cancer for metastatic workup , no available previous examination

I- BRAIN:

- Left cerebellar space occupying lesion is seen measures about 3.2 x 2.3 cm in axial dimensions showing thick marginal enhancement and surrounded by perifocal edema and mass effect in the form of compression of the posterior aspect of brain stem.
- Moderate dilatation of supra-tentorial ventricular system with no significant midline shift or deformity.
- Prominent cortical sulci and extra axial CSF spaces.
- Preserved normal gray-white matter interface.
- No extra-axial or intra-cerebral areas of fresh blood densities.

OPINION:

- *Left cerebellar SOL likely metastatic.*
- *Brain involutional changes with no evidence of ischemic vascular insults.*

II- CHEST:

- Right MRM showed clear operative bed with no evidence of enhanced focal lesion or fluid collection.
- Bilateral basal lung lobes atelectatic bands are noted.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- The heart is seen enlarged in size.
- Left 4th rib healed fracture is noted
- Small hiatus hernia.

III- ABDOMEN AND PELVIS:

Patient Name: Nasra Abd Al Baset Emam Al Sayed	Date : 20/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 64 Year(s)
Medical ID: 1519/2021	Pat. ID: 17011

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of managed right breast cancer for metastatic workup , no available previous examination

I- CHEST:

- Right MRM showed clear operative bed with no evidence of enhanced focal lesion or fluid collection.
- Bilateral basal lung lobes atelectatic bands are noted.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- The heart is seen enlarged in size.
- Left 4th rib healed fracture is noted
- Small hiatus hernia.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
 - The right hepatic lobe segment VII showed ring enhanced focal lesion measures about 1.6x1 cm
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Apart from right lower cortical simple cyst measures about 6mm
- Bilateral bulky suprarenal glands
- Normal C.T appearance of the spleen, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined bones showed dorsolumbar spondylosis with no worrisome lytic or sclerotic lesions.

Name : Nasra Abdel Baset

Date : 07-09-2026 12:33

Branch : Dokki

Medical No. : 456344

Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE BONE SCAN REPORT

65-year-old female with right breast carcinoma, status post right modified radical mastectomy (~5 years ago). Referred for routine follow-up skeletal evaluation compared to prior bone scan dated November 26, 2025.

Relevant prior CT (June 9, 2026) showed a solitary expansile lytic lesion in the left 3rd rib without cortical destruction.

FINDINGS:

- Symmetrical physiological tracer distribution throughout the skull, spine, ribs, sternum, and pelvis.
- Complete resolution of the previously reported focal uptake at the lateral aspect of the left 4th rib is noted.
- Mild irregular non-specific uptake is noted in the spine and sacroiliac joints, consistent with multilevel degenerative spondylotic changes.
- Persistent very faint focal uptake is noted at the distal shaft of the left femur, noticeably decreased in intensity compared to the prior study.
- Bilateral shoulder, knee, and ankle joints show mild-to-moderate uptake, consistent with degenerative arthritic changes.
- The left 3rd rib lytic lesion seen on CT shows no corresponding tracer uptake.

Name : Nasra Abdl Baset

Date : 07-09-2026 12:33

Branch : Dokki

Medical No. : 456344

Ref.Physician Prof : Goustav Roussey Egypt

IMPRESSION:

- No definite scintigraphic evidence of active osteoblastic metastases on the current study.
- Persistent faint left femoral uptake is atypical for active metastasis and likely benign/traumatic; for targeted imaging.
- The left 3rd rib lytic lesion shows no scintigraphic uptake.
- Clinical correlation and oncology review are advised.

Much oblige

Dr. Alaa Nagi, MD

Nuclear medicine

Patient's Name:

Nasra Abdelbasset Imam Al-Sayed

Date:

2026-09-14

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Exam:

Multidetector CT Examination of the Chest, Abdomen, and Pelvis with Intravenous Contrast

Clinical History:

Known breast carcinoma, status post right-sided mastectomy. Follow-up for evaluation of metastatic disease.

Technique:

Multidetector CT examination of the chest, abdomen, and pelvis was performed following intravenous contrast administration. Multiplanar reformatted images were obtained.

Comparison:

Prior CT Chest, Abdomen, and Pelvis dated 09/06/2026.

Findings:

Chest:

Postsurgical changes of right-sided mastectomy are noted, without definite residual or recurrent enhancing soft-tissue mass within the mastectomy bed or adjacent chest wall.

Newly visualized mildly enlarged left axillary lymph node measuring approximately 18 × 9 mm, favored reactive. No pathologically enlarged right axillary, supraclavicular, mediastinal, or hilar lymph nodes.

Mild diffuse bronchial wall thickening with scattered small dependent/gravitational ground-glass and linear patchy opacities, likely reflecting mild inflammatory/atelectatic changes. No focal pulmonary mass, suspicious pulmonary nodule, or lobar air-space consolidation.

Minimal pericardial effusion is noted. Mild cardiomegaly. No pleural effusion or pneumothorax.

Bilateral infrascapular soft-tissue masses with typical appearance of elastofibroma dorsi.

Abdomen and Pelvis:

Small peripherally enhancing focal lesion in hepatic segment VII measuring approximately 1.2 × 0.8 cm, indeterminate in the current examination. Differential considerations include atypical hemangioma and metastatic deposit. No additional definite focal hepatic lesions. No intrahepatic or extrahepatic biliary ductal dilatation.

Gallbladder, pancreas, spleen, adrenal glands, and both kidneys show no significant focal abnormality. No hydronephrosis.

Patient's Name:

Nasra Abdelbasset Imam Al-Sayed

Date:

2026-09-14

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

No significant abdominal or pelvic lymphadenopathy. No ascites, peritoneal thickening, or omental nodularity.

Urinary bladder is unremarkable. No definite pelvic mass or free pelvic fluid.

Osseous Structures:

Stable solitary expansile lytic lesion involving the left third rib without significant interval cortical destruction. In the setting of known malignancy, osseous metastatic involvement remains a consideration.

Mild multilevel degenerative spondylotic changes. No acute or pathological fracture identified.

Impression:

1. Postsurgical changes of right mastectomy without definite CT evidence of local tumor recurrence.
2. Newly visualized mildly enlarged left axillary lymph node (18×9 mm), favored reactive; attention on follow-up is recommended.
3. Mild bronchial wall thickening with scattered dependent/gravitational ground-glass and atelectatic changes, likely inflammatory/atelectatic.
4. Indeterminate 1.2×0.8 cm peripherally enhancing segment VII hepatic lesion; metastatic deposit cannot be excluded. Dedicated contrast-enhanced liver MRI is recommended for characterization. Stable left third rib expansile lytic lesion, suspicious for osseous metastatic involvement.

Ibrahim

Name : Nasra Abdel Baset

Date : 07-09-2026 12:33

Branch : Dokki

Medical No. : 456344

Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE BONE SCAN REPORT

65-year-old female with right breast carcinoma, status post right modified radical mastectomy (~5 years ago). Referred for routine follow-up skeletal evaluation compared to prior bone scan dated November 26, 2025.

Relevant prior CT (June 9, 2026) showed a solitary expansile lytic lesion in the left 3rd rib without cortical destruction.

FINDINGS:

- Symmetrical physiological tracer distribution throughout the skull, spine, ribs, sternum, and pelvis.
- Complete resolution of the previously reported focal uptake at the lateral aspect of the left 4th rib is noted.
- Mild irregular non-specific uptake is noted in the spine and sacroiliac joints, consistent with multilevel degenerative spondylotic changes.
- Persistent very faint focal uptake is noted at the distal shaft of the left femur, noticeably decreased in intensity compared to the prior study.
- Bilateral shoulder, knee, and ankle joints show mild-to-moderate uptake, consistent with degenerative arthritic changes.
- The left 3rd rib lytic lesion seen on CT shows no corresponding tracer uptake.

Name : Nasra Abdl Baset

Date : 07-09-2026 12:33

Branch : Dokki

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IMPRESSION:

- No definite scintigraphic evidence of active osteoblastic metastases on the current study.
- Persistent faint left femoral uptake is atypical for active metastasis and likely benign/traumatic; for targeted imaging.
- The left 3rd rib lytic lesion shows no scintigraphic uptake.
- Clinical correlation and oncology review are advised.

Much oblige

Dr. Alaa Nagi, MD

Nuclear medicine

Patient's Name:

Nasra Abdelbasset Imam Al-Sayed

Date:

2026-09-14

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Exam:

Multidetector CT Examination of the Chest, Abdomen, and Pelvis with Intravenous Contrast

Clinical History:

Known breast carcinoma, status post right-sided mastectomy. Follow-up for evaluation of metastatic disease.

Technique:

Multidetector CT examination of the chest, abdomen, and pelvis was performed following intravenous contrast administration. Multiplanar reformatted images were obtained.

Comparison:

Prior CT Chest, Abdomen, and Pelvis dated 09/06/2026.

Findings:

Chest:

Postsurgical changes of right-sided mastectomy are noted, without definite residual or recurrent enhancing soft-tissue mass within the mastectomy bed or adjacent chest wall.

Newly visualized mildly enlarged left axillary lymph node measuring approximately 18×9 mm, favored reactive. No pathologically enlarged right axillary, supraclavicular, mediastinal, or hilar lymph nodes.

Mild diffuse bronchial wall thickening with scattered small dependent/gravitational ground-glass and linear patchy opacities, likely reflecting mild inflammatory/atelectatic changes. No focal pulmonary mass, suspicious pulmonary nodule, or lobar air-space consolidation.

Minimal pericardial effusion is noted. Mild cardiomegaly. No pleural effusion or pneumothorax.

Bilateral infrascapular soft-tissue masses with typical appearance of elastofibroma dorsi.

Abdomen and Pelvis:

Small peripherally enhancing focal lesion in hepatic segment VII measuring approximately 1.2×0.8 cm, indeterminate in the current examination. Differential considerations include atypical hemangioma and metastatic deposit. No additional definite focal hepatic lesions. No intrahepatic or extrahepatic biliary ductal dilatation.

Gallbladder, pancreas, spleen, adrenal glands, and both kidneys show no significant focal abnormality. No hydronephrosis.

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No significant abdominal or pelvic lymphadenopathy. No ascites, peritoneal thickening, or omental nodularity.

Urinary bladder is unremarkable. No definite pelvic mass or free pelvic fluid.

Osseous Structures:

Stable solitary expansile lytic lesion involving the left third rib without significant interval cortical destruction. In the setting of known malignancy, osseous metastatic involvement remains a consideration.

Mild multilevel degenerative spondylotic changes. No acute or pathological fracture identified.

Impression:

1. Postsurgical changes of right mastectomy without definite CT evidence of local tumor recurrence.
2. Newly visualized mildly enlarged left axillary lymph node (18×9 mm), favored reactive; attention on follow-up is recommended.
3. Mild bronchial wall thickening with scattered dependent/gravitational ground-glass and atelectatic changes, likely inflammatory/atelectatic.
4. Indeterminate 1.2×0.8 cm peripherally enhancing segment VII hepatic lesion; metastatic deposit cannot be excluded. Dedicated contrast-enhanced liver MRI is recommended for characterization. Stable left third rib expansile lytic lesion, suspicious for osseous metastatic involvement.

Ibrahim